

They'll be Protected in Colorado: Reproductive Health Access Organizations' Use of Technology

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Abstract

In the wake of the *Dobbs v Jackson* decision—and the subsequent limits on abortion access—the use of technologies to coordinate access to abortion care in the United States has evolved. Organizations which have coordinated access to abortion for decades are increasing their use of technologies to address the gaps in care which are emerging from the complex legal landscape. This work in progress examines the ways that abortion access organizations are caring for abortion seekers, and using activism to make that care possible. We conducted a qualitative interview study with 11 abortion access workers in Colorado and 2 in national telehealth which serve Colorado. In our analysis, we show the relationships between abortion access workers and the technologies they use. We preview interviews in-progress, and anticipate future work to design resources for abortion access organizations to bring into being a reproductive justice future, through both care and activism.

CCS Concepts

• Human-centered computing → Empirical studies in HCI.

Keywords

abortion, care, activism, reproductive justice

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1 Introduction

The ability to make decisions about one's own body, including the choice to terminate a pregnancy, is integral to upholding human rights and individual agency [84]. To support people in exercising their reproductive agency, a wide array of organizations collaborate to aid people seeking abortions¹ [80]. These organizations provide

¹We refer to people who are pregnant and seeking abortions as 'abortion seekers', 'pregnant people' and 'patients'. While historically abortion has been framed as a women's issue, we are intentionally choosing a gender neutral appellation to acknowledge the wide gender diversity of people who seek abortions. While not all organizations we

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financial, logistical, and emotional support for abortion seekers [57]; fight to combat restrictive laws [33]; and ensure safer abortions can be accessed without legal consequences [78]. Individuals working for abortion access within these organizations both care for people seeking abortions and advocate for positive changes in the abortion space.

In the United States, since *Roe v Wade* [58] was overturned, abortion has increasingly been legally restricted or banned across different states, with a growing number of people needing to travel to access abortions. This has intensified challenges of coordinating abortion access due to increased cost, logistical complexities, and deliberately confusing and opaque legal restrictions [72]. As more people travel to access abortions and shifting legal contexts obfuscate paths to abortion care, abortion access networks face unprecedented demand. In response, abortion access organizations have turned to technologies to continue providing care. These technologies, such as chat applications, online intake forms, databases, and messaging platforms, are impacting the coordination of abortion access workers as they support abortion seekers. Though previous HCI scholarship has shown recent interest in the use of technology for abortion related issues [48, 50, 51, 56], no work thus far has looked at the organizations coordinating abortion care.

Understanding the coordination of abortion care is increasingly vital as abortion access and reproductive health care will be under greater threat in the coming years. After the 2024 presidential election in the United States, anti-abortion rhetoric surged on social media, with anti-abortion individuals, white nationalists, and men's rights activists celebrating the election as a victory for "traditional values". Messages such as "your body my choice", "we own your body", and "get back in the kitchen" [21] proliferated online. With the new administration sworn in, the critical role of technology in abortion access has been made especially clear: for instance, the federal website *reproductiverights.gov*, which had previously provided information about abortion and contraception, was immediately taken down after the inauguration [24]. Additionally, abortion-related organizations have begun to face censorship on social media sites [81].

To explore the role of technology in abortion access coordination, as well as how designers might support that organizing in the face of rising restrictions, we conducted semi-structured interviews with 11 abortion access workers in the state of Colorado, and 2 telehealth providers who served Colorado. In this work in progress,

interviewed with were medical in nature, 'patients' is the language most participants used, and we will use for consistency throughout the paper, along with 'pregnant people' and 'abortion seekers'.

we investigate how abortion access workers and their organizations use technologies to coordinate with one another and provide support for abortion seekers. We focused the study on Colorado in this stage of the project due to its position as a receiving state for abortion seekers (meaning people travel from states with restrictions to access care in Colorado), and its proximity to restrictive abortion states. In this study we ask:

- (1) *How do abortion access workers and organizations coordinate to support abortion seekers?* and
- (2) *What are the design priorities for communication and organization technologies to better support abortion access workers and organizations?*

We found that abortion access workers were connected across local and national contexts, using a combination of organization and communication technologies to maintain those connections. Abortion access workers utilized technologies for their speed, ease of use, and security features. We found that the work of abortion access was interconnected across state borders, organizations, and technologies, and that abortion access workers emphasized their work as both care for abortion seekers and activism for a more just reproductive health future.

2 Background

On June 24th, 2022, the Supreme Court of the United States turned legality of abortion back to the states with the *Dobbs v Jackson* decision [59]. At the time of writing, abortion is banned in 13 of the 50 states, and 7 states have limits on how late in pregnancy one can get an abortion [34].

Due to restrictions, pregnant people in states with abortion bans have to travel to states with protected access such as Colorado, facing significant barriers and emotional distress [65]. States where abortion is legally protected have seen a significant influx of patients seeking care after the *Dobbs v Jackson* decision [39]. Colorado is one such state, where abortion access is enshrined in the state constitution, and abortion providers are protected from prosecution [4].

Abortion access organizations play a critical role in addressing the heightened gaps in care, particularly in states like Colorado, where many travel for abortion services. Abortion access organizations support abortion seekers in their care journey from home to clinic and back [20]. Importantly, these organizations consider abortion as one part of **reproductive freedom**, a concept created by Black feminist activists to call to attention the intersecting systemic oppressions involved in reproductive autonomy [67]. Therefore, abortion access organizations include care for abortion seekers comprehensively, not just for their appointment. This includes paying for their appointments, booking flights and hotels, arranging child care, and providing emotional support throughout their journey. Definitions of the varying types of abortion access organizations that will be discussed in this paper are available in Appendix A.

3 Related Work

To motivate and contextualize our research, we first discuss the gap in research on abortion in HCI. We then examine theories of care and activism, explore how HCI has built on these theories, and

identify areas where our study contributes to and expands existing work.

3.1 Reproductive Health and Abortion Research in HCI

In the face of increased restrictions to abortion care, abortion access organizations are leaning on technologies to help them bridge gaps in care. Examples include online chatbots that assist with information gathering about abortions [31], mobile applications that provide support for abortion seekers and providers [76], and websites that help abortion seekers find clinics and navigate changing legal landscapes [19]. These technologies have been essential to supporting abortion care.

Thus far, very little work in HCI has attended to the role of technology in abortion care. Lusi et al. [50] proposed a prototype of an after-abortion mental self-care package, and Calkin [7] discusses the merits of abortion care via telehealth. Beyond these two examples, abortion research in HCI has focused on the political side of abortion such as fighting stigma through abortion storytelling [44, 53], discussions of abortion policy on social media sites [30, 71], political activism [26, 48], and privacy concerns with the criminalization of abortion [51]. Notably, no prior HCI research has examined the role of abortion access organizations or their use of technologies to coordinate care.

Other HCI research on reproductive health has attended to care provision and empowering women to care for their reproductive health, from postnatal experiences [17, 18] and breastfeeding [83, 88], to menstrual [8, 9, 13, 49] and urinary health [3]. Similar to the few papers on abortion care, most studies on reproductive health in HCI focus on the intimate relation between a person and their reproduction, rather than care organizations.

Abortion, as a uniquely politicized and moralized form of care [90], presents unique challenges. To address these challenges, our work engages with studies of both care and activism in HCI. In the following sections, we review prior work on community-based care and activism.

3.2 Community Care in HCI

Studies on care practices have highlighted the creation and upkeep of care networks as a crucial strategy during crisis situations [6, 22, 41, 87]. Much of this research focuses on community responses to acute moments of crisis such as a natural disaster [22, 60, 61, 73]. Knearmen et al. [40] discuss the importance of building community strength in the context of COVID-19 through ‘care-mongering’, which serves as a strong example of prolonged crisis and the role of technology and community in responding to it. The state of abortion access in the United States, while not officially considered a crisis by governing bodies, is considered a long-term crisis by activists [62], human rights organizations [84, 86], and journalists [14, 80] based on the mortal consequences and quality of life impact it is having on those denied care.

Work in HCI has also interrogated the tensions between use of technologies and the provision of care. Rossitto et al. [68] explored the challenges in balancing the efficiencies afforded by technologies with the messy, embedded commuting of care. Other work has problematized the use of care in the healthcare field, creating

challenges for care providers and interrupting their connections to patients [35, 77]. Therefore, introducing technologies to address gaps in care also introduce challenges to providing good care [70].

3.3 Activism in HCI

Theories of activism in HCI emphasize change and transformation rather than maintenance, as activist movements produce new values and transform societal norms [10]. Digital activism has amplified collective action by mobilizing large groups and fostering networks of actors, including people, organizations, and technology [23, 52, 54]. Coalition building protest movements such as the Anti-ELAB movement in Hong Kong² [43] and the Occupy Wall Street protests³ [2, 37, 66] show how technologies mediate activist networks and sustain movements by fostering connections and coalitions across boundaries [42, 74, 75, 82].

While care and activism have been studied separately within HCI, both intersect in their exploration on the kinds of worlds created by the technologies we maintain. Both care and activism also operate within networks of people working toward change. In this work in progress, we contribute to HCI research on reproductive health by examining how abortion access organizations work through technologies. Our findings highlight the intersections of care and activism and provide research implications for studying abortion access in HCI, and design implications for supporting abortion organizing.

4 Methods

We conducted a qualitative interview study with abortion access organizations in Colorado, focusing on their interactions with other organizations and their use of technology. Data collection occurred between May 2023 and October 2023, coinciding with the one-year anniversary of the *Dobbs v Jackson* decision. During this period, significant court and legislative decisions around the legality and protected status of abortion shifted across the country. This research was approved by two separate university ethics boards, one in the United States, and one in Canada.

Recruitment was done using a combination of snowball sampling [29] from the lead researcher's personal network in abortion activism and cold-contacts using publicly available information. Given the sensitive nature of abortion access work, leveraging the lead researcher's personal network was essential to establish trust and recruit participants. We interviewed 13 abortion access workers from various organizations to represent the diversity in roles included in the abortion access network. The specific roles of interviewees are listed in Table 1. Note that the clinic defenders (CD) 1-5 were interviewed together. The majority of our participants were women (12/13), and one non-binary person. These demographics align with broader trends in reproductive justice activism, where women are over-represented and are disproportionately affected by reproductive justice issues [11, 67].

²The Anti-Extradition Law Amendment Bill (ELAB) Movement was a series of protests in Hong Kong in 2019-2020 [43].

³Occupy Wall Street was a series of protests and sit-ins which started in New York City, United States. They focused on economic inequality and corporate greed, particularly after the "great recession" in the United States [66]

P#	Organization	Gender
P1	Citizen Activist and Clinic Defender	Woman
P2	Political Organization	Woman
P3	Telehealth Abortion Fund	Woman
P4	Abortion Clinic	Woman
P5	Local Abortion Fund	Woman
P6	Political Organization	Woman
P7	Telehealth Abortion Provider	Non-Binary
P8	Political Organization	Woman
CD 1-5	Clinic Defenders	Woman

Table 1: Participant Information

4.1 Interviews and Data Analysis

The lead researcher conducted qualitative semi-structured interviews to gain insight into the perspectives and largely undocumented, experiences, expertise and efforts of abortion access workers. The interviews focused on their coordination work and technology use in the state of Colorado. Interviews were conducted both in-person and online, according to the comfort of the interviewee. The interviews were audio-recorded with the consent of participants, and transcribed by a locally-installed secure transcription tool.

We analyzed the interview data inductively, using Braun and Clarke's thematic analysis approach [15], emphasizing iterative coding and thematic development. First, each member of the research team independently read through all interview data. Each research team member built an initial list of codes based on the study's research questions. Next, the research team met, discussed the initial codes, and iteratively revised the codes over the course of multiple meetings. The research team then used this code list to code all interviews. After coding was complete, researchers identified themes independently, and then in discussion with the research team. Researchers discussed areas of disagreement until consensus was reached and themes finalized. Notably, consensus in this context does not mean that all researchers perfectly aligned on their understanding of the data. Rather, it means that all researchers agreed on their understanding of themes to facilitate analysis of the data. From the final theme set, we discuss three major themes: interdependence in abortion access, privacy and security, and reproductive futures.

5 Findings

Through our interviews, we found that abortion access workers and the organizations that are part of their network maintain complex relationships with, and perspectives on, the use of technologies. This section explores the technologies abortion access workers use to communicate and coordinate with one another and with abortion seekers. These technologies include end-to-end encrypted messaging services, cloud collaboration data management software, phone, and text. We also outline the unique interdependent relationship between care work and activism in abortion access.

5.1 Technology Use in Abortion Access Organizing

The work of abortion access operates within a complex web of interdependent actors and organizations. We found that not only were a variety of organizations involved in the abortion access network, but individual actors would fill multiple roles depending on the needs of the moment. As one participant reported when discussing the many roles they play in abortion access: *"I feel like once you dip your toe in abortion, it just spirals and now you're a jack of all trades"* (P7). All abortion access workers in the network worked tirelessly to care for *"whatever that patient needs, they will bend over backwards and make it happen as long as they're able to. They've got a lot of wonderful people there that will follow the patient through their whole process"* (P4).

Abortion access organizations coordinated through digital technologies to ensure comprehensive care for abortion patients throughout their entire journey, from organizing funding to accompanying the abortion seeker at their appointment. As P4 reports, *"booking flights, hotel accommodations, rides... I've coordinated childcare for folks before, so really whatever they need, people find some resources for it."*

The use of different technologies was shaped by the varying legal and financial contexts of abortion funds, which also impacted how they provided funds for patients. When an abortion fund did not have sufficient funding to cover the patient, they activated their connections to other local and national funds using communication technologies such as Slack and phone calls, and via online forms such as Google forms and Microsoft forms. Additionally, if a patient applied to one abortion fund but did not meet eligibility requirements, rather than requiring the patient to find financial assistance somewhere else, the fund would coordinate with other local and national funds to put together the needed money, either through online intake forms or personal contacts on Signal or text message.

5.1.1 Rapid Communication and Time-Sensitive Care. The time-sensitive nature of abortion care heightened the importance of rapid communication among abortion access workers. While online forms facilitated formal coordination, informal communication channels such as Signal chats or direct phone calls were used for cases in which a rapid response was vital. P4 discussed how she used Signal chats to quickly coordinate funds with the local abortion fund rather than go through their online form: *"Usually I will say something about, you know, hey, we have a patient scheduled tomorrow and I'm looking to coordinate last minute travel assistance for them."* Due to the time-sensitive nature of abortion care, this rapid, personal communication is essential.

Clinic defenders also played a critical role in abortion access by ensuring abortion seekers would feel comfortable as soon as they arrived at the clinic, despite anti-abortion protesters establishing an *"intimidating presence outside of the clinic"* (P1). As CD1 explained, *"You know we're gonna get you in here safe"*. Clinic escorts used text messaging to coordinate their schedules to ensure there was never a day that patients were left undefended at the clinics as text messaging was quick and simple for them to use (CD1-5).

5.1.2 Tracking and Managing Care Networks. To keep track of the network of actors at play for abortion access, interviewees maintained detailed records, either publicly or privately, of their partnerships. For instance, P4 described their list of social and emotional resources for patients: *"It's spreadsheets, really. Yeah. Yeah. Lots of spreadsheets. I've got a spreadsheet for abortion funds and travel funds and then community resources as well, which is something I'm looking to really improve on."* (P4). For spreadsheets, organizations primarily used cloud collaboration software, such as Airtable and Google Sheets. P5's organization was similarly working on a repository of organizations to document resources for patients, and P8 expressed a desire to creating one as well.

5.1.3 Maintaining Safety and Privacy. Given the political and controversial nature of abortion, managing visibility of the abortion access network was important for our interviewees. The threat of anti-abortion actors targeting abortion access workers and patients also impacted their choices of technologies to coordinate access. P4 noted that Signal was the preferred format for these communications because it's *"HIPAA compliant"*. This was especially important for patients seeking care from out of state, as encrypted messaging services have better confidentiality of information. However, not all workers consistently adopted these platforms. P8 explained how her colleagues got on Signal but quickly abandoned it. P5 highlighted the need for a service that combines the user-friendliness of Slack with adequate security features.

For people unable or unwilling to access abortion care in-person due to security or privacy concerns, telehealth has increasingly become a preferred option of accessing care. Telehealth allows patients to avoid travelling, and can be a more private experience than having an in-clinic abortion: *"You're not seeing somebody, people are asking you, texting you questions, you get to reply. And that's how a lot of care is done for these folks. And so a lot of people like that anonymity of, of that care"* (P3). To further protect patient privacy, abortion funds associated with P3 and P7 only collect initials from applicants, minimizing the risk.

5.2 Political Action

Political organizations worked alongside abortion funds, clinics, and practical support organizations to advocate for legislation protections that enable abortion access organizations to function. Political organizing for abortion access is particularly important in Colorado, where people travel from banned states to seek care. As P2 explained: *"We got legislation through that says, well, one, we won't expedite you or extradite you...They'll be protected in Colorado."*

Due to the evolving legal landscape, and the heightened need for privacy during political campaigns, political actors relied on a combination of text and encrypted messaging for more personal and rapid communication. For example, P8 uses Signal for communication that is *"Rapid. I'm at the [state] capital. I have a question I need an answer"* and P2 *"prefer(s) text. It's the fastest"*.

Similar to the work of abortion funds, this more casual, rapid, and personal communication style reflects the tight-knit nature of the abortion space: *"I've noticed that a lot of people would be like, oh, yeah, just have this person's phone number or like, we know each other. Like, I feel like part of it is probably because it's just like a small space in general. And so folks know each other"* (P4). These informal,

rapid communication practices highlight the interconnectedness of abortion access networks and how crucial it was personally knowing others. This further shaped the trust that existed in the space and the choice of technologies.

5.3 Need for Inclusion

The relationship of queer identities to abortion was also central to our interviews, as many anti-trans and anti-LGBTQIA2S+ bills have been partnered with anti-abortion legislation [12]. As noted by P7, who identified as non-binary, *"it's still very like woman this and woman that and I think it's a really hard thing as a non-binary person I'm like, I both don't want that gendered language but also see that if that can get some moderate people to vote in a certain way that it has a purpose and I don't know what the answer is there how we balance all of that"* (P7). Some participants focused strongly on women in particular, *"It feels... for one thing, there's a war on women going on. The GOP⁴ is losing control and they want to control women"* (CD3).

For the people we interviewed, issues of abortion access were a component of a much larger liberatory effort within the diverse goals of reproductive health, particularly issues which impact marginalized populations' access to reproductive autonomy. Part of the reason this is the case could be that these organizations take a reproductive justice approach to abortion care which includes *"bodily autonomy and self-determination... queer liberation, trans liberation"* (P8). This means that while many of them are explicitly abortion related, they see other social issues as intrinsically tied to abortion and reproductive care. Abortion work was not only networked within the various organizations, but also to other reproductive health and political initiatives.

6 Discussion

6.1 Preliminary Suggestions for Supporting Abortion Care Technology Networks

To develop technology systems that more effectively support abortion access networks, we argue that HCI adopt research and design approaches that: (1) embrace flexibility and privacy, (2) address sociocultural concerns, and (3) attend to the personal and political connections in abortion access. While these approaches are fairly general and lack concrete guidance, we anticipate refining this guidance in future work, as detailed in 6.2.

6.1.1 Flexibility, Speed, and Privacy. Designing communication technologies to support abortion access requires balancing flexibility, privacy, and speed. Privacy is particularly critical in the abortion space, where patients and workers face surveillance, legal risks, and personal safety threats. However, overly complex privacy-focused technologies can hinder care work, and slow down the communication flow by imposing a steep learning curve or disrupting workflows. Participants in our study prioritized maintaining their networks and providing warmth and support to patients and ensuring rapid care over digital security expertise. Future designs must avoid disrupting existing systems and prioritize the relational and emotional dimensions of care, which rely on embedded expertise,

strong connections, and flexible back channels to provide patient-centred care without jeopardizing security or trust [35, 64, 68].

6.1.2 Sociocultural Concerns. Future design for abortion care also requires attending to the complex, sociopolitical nature of care work and advocacy in the abortion space. Drawing on Tronto's [79] perspective, care is a relational, community-based action tied to sociopolitical issues [32, 45, 87]. Abortion workers view their work as interconnected with reproductive justice and other social causes such as trans rights, highlighting the intersectional nature of abortion access. Technologies supporting abortion activism must reflect diverse cultural and personal needs, addressing issues like racial and trans/non-binary justice, language barriers, and harmful stereotypes [11, 55].

6.1.3 Personal and Political Connections in Abortion Access Work. Abortion workers' work was entangled with the work of other organizations, legal landscapes, and systemic barriers to reproductive care, demonstrating resistance to power structures and fostering a community of mutual support [69]. This networked activism defies state-by-state limitations on abortion access, emphasizing shared responsibility and collective care, aligning with work which highlights the moral imperative to care.

Abortion access activists formed a flexible, distributed network of care reminiscent of other forms of grassroots activism [2, 16, 74, 75]. Unlike centralized technologies often used in medical care [27, 28, 85], abortion access work relied on diverse technologies to address patient needs while evading hostile systems [1, 38, 65].

The roles of actors in Colorado's abortion access network demonstrate the co-existence of care and activism in reproductive justice. For HCI researchers, designing technologies for supporting reproductive health organizations involves understanding these dual roles. By embedding these values into design, HCI researchers can create tools that support the intersecting goals of care and activism in abortion access networks.

6.2 Limitations and Future Work

The research herein has some limitations which open up opportunities for future work. First, this research only looked at the context of Colorado during 2023. Focusing our data collection on a single state brought depth to our findings but also meant that some specifics, such as the network structure, cannot be generalized to other places in the United States or other countries. Second, we did not interview abortion seekers. It is important to know how the abortion access network impacts abortion seekers from their perspective and for design to consider their needs and experiences. Further, drawing on calls for researchers to become activists themselves [5, 47, 89], we believe that a limitation of this study is that our research did not immediately result in a specific artifact or political action that supports abortion access. To address some of these limitations, we are conducting continuing work on abortion access organizing.

Currently, the research team is conducting interviews in Illinois (which has protective measures and is a receiving state for abortion care, similar to Colorado) to expand the scope of this work and respond to significant changes in the abortion landscape since 2023, and to cover a more diverse geography. Through these interviews

⁴The Grand Old Party (GOP) is the conservative republican party in the United States.

we will ask how the increasing criminalization of abortion is changing the way that abortion access organizations are able to form alternative care networks. Due to the hostility of the current federal government to abortion and heightened misogynistic rhetoric, we are interested in how the abortion activism networks respond to the shifting context.

To the end of implementing our insights from this study into more actionable support, our future work will include participatory workshops with abortion access workers with the intent of creating resources to support abortion access organizations. We will design these workshops based on the results from the research described in this late breaking work, as well as our further work in Illinois. Through these workshops we aim to support abortion access organizations in laying the groundwork for a reproductive justice future.

Acknowledgments

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A Organizational Definitions

To source the definitions below, we used resources from national organizations' websites such as the National Network of Abortion Funds [57], Apiaray for Practical Support [25], Hey Jane [36], and Planned Parenthood [63].

Abortion Clinics: Abortion clinics are medical facilities specifically designed to provide abortion services, a medical procedure for terminating a pregnancy. Staffed by trained healthcare professionals, these clinics offer a range of reproductive health services, including counselling, examinations, and procedural (performed by a physician) or pill-based abortions. They may also offer post-abortion care and related reproductive health services as part of their comprehensive healthcare approach.

Abortion Funds: Abortion funds are organizations that provide financial assistance to individuals who cannot afford the costs associated with obtaining an abortion. These funds aim to eliminate economic barriers to accessing reproductive healthcare, covering expenses such as the procedure itself, travel, and accommodation.

Volunteers and grassroots efforts are often integral to the functioning of these funds, which may operate on a local, regional, or national level.

Clinic Defenders: Abortion clinic defenders, also known as escorts, volunteer to provide protection and support to individuals seeking reproductive healthcare at abortion clinics, particularly abortions. Their primary role is to help patients navigate through anti-abortion protests and potential harassment outside clinics, ensuring a safe and supportive environment [46].

Telehealth Abortion Providers: Telehealth abortion is a method that allows individuals to receive abortion services remotely through telecommunication technologies. Healthcare providers prescribe medication, specifically mifepristone and misoprostol, for patients to take at home under their medical guidance. Telehealth abortion is valued for its potential to improve access to safe and legal abortion services. For example, during the COVID-19 pandemic, telehealth abortions were vital to ensure continued access despite closures of abortion clinics.